

Foundation of Special and Inclusive Education

Learning Module No. 01

TEACHER
Online Review Center

STUDENT

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LEARNING MODULE INFORMATION

I. Course Code	
II. Course Title	Foundation of Special and Inclusive Education
III. Module Number	01
IV. Module Title	Special Education (SPED) in the Philippines
V. Overview of the Module	This module discusses the vision, policy, goal and objectives, programs and services and meaning of special education in the Philippines. Also, this includes the advocacy of all the children with special needs, students with mental retardation, and learning disabilities,
VI. Module Outcomes	At the end of this module, students will be able to explain how the government promotes SPED, explain the vision of the government for children with special needs, cite policies of SPED as well as its goals and objectives, enumerate milestones in the history and development of SPED from 1907 up to present, and appreciate inspiration from the lives of successful persons with disabilities.
VII. General Instructions	The module is designed to assess student understanding of the assigned lessons found within the associated content of the preliminary term, midterm, and final term periods of the course. The assessment part of the module is composed of varied types of questions. You may see true/false, traditional multiple choice, matching, multiple answer, completion, and/or essay. Pay attention to the answer to the assessment questions as you move through each lesson. After each module you will be given a summative test. Your responses to the assessment parts of the module will be checked and recorded. Because the assessment questions are available within the whole completion period and because you can reference the answers to the questions within the content modules, we will not release the answers within modules. However, your professors are happy to discuss the assessments with you during their consultation time, should you have any questions. Good luck. You may not work collaboratively. This is independent work.

Lesson 1: Special Education (SPED) in the Philippines

Special Education is an educational program/service designed to meet the needs of children with special needs who cannot profit from general or regular education because of disabilities or exceptional disabilities. It is derived from the premise that in a democracy, every individual is valuable in his own right and should be afforded equal opportunities to develop his full potential. Equal educational opportunities do not mean the same educational experiences, but rather "different" experiences based on the child's unique needs. The right to education cannot be denied a person if only because of his disabilities.

Lesson Objectives:

1. Identify different categories of children and youth with special needs;
2. Explain the vision for children with special needs;
3. Discuss the policy of Inclusive Education for All;
4. Enumerate the goal and objectives of SPED;
5. Discuss legal bases of SPED in the country;
6. Define the following terms: prevalence, identifiable prevalence, true prevalence, incidence;
7. Compare the prevalence estimate of children with special needs done by the UNICEF and the World Health Organization;
8. Explain the figure on the true prevalence of Filipino children and youth with special needs;
9. Describe the different special education programs and services offered by the Philippine public and private schools or institutions and cite examples for each;
10. Discuss the definition of inclusive education and its salient features;
11. Enumerate the support services extended to children with special needs;
12. Define SPED;
13. Explain how SPED enables exceptional children to benefit from basic education program of DepEd;
14. Define children with special needs (CSN);
15. Distinguish basic terms in SPED; and
16. Develop positive attitude towards exceptional children and youth.

Getting Started (Optional):

How can we strengthen the teaching facility in government schools for learning disabled children?

Discussion:

Vision for Children with Special Needs

The Department of Education clearly states its vision for children with special needs in consonance with the philosophy of inclusive education, thus:

"The State, community and family hold a common vision for the Filipino child with special needs. By the 21st century, it is envisioned that he/she could be adequately provided with basic education. This education should fully realize his/her own potentials for development and productivity as well as being capable of self-expression of his/her rights in society. More importantly, he/she is God-loving and proud of being a Filipino.

The policy on Inclusive Education for All is adopted in the Philippines to accelerate access to education among children and youth with special needs.

It is also envisioned that the child with special needs will get full parental and community support for his/her education w/o discrimination of any kind. This special child should also be provided with a healthy environment along with leisure and recreation and social security measures" (Department of Education Handbook on Inclusive Education,2000).

Policy, Goal and Objectives of SPED

The policy on *Inclusive Education for All* is adopted in the Philippines to accelerate access to education among children and youth with special needs. Inclusive education forms an integral component of the overall educational system that is committed to an appropriate education for all children and youth with special needs.

The goal of the special education programs of the Department of Education all over the country is to provide children with special needs appropriate educational services within the mainstream of basic education.

The two-pronged goal includes the development of key strategies on legislation, human resource development, family involvement and active participation of government and non-government organizations.

Likewise, there are major issues to address on attitudinal barriers of the general public and effort towards the institutionalization and sustainability of special education programs and services.

1. Provide support services, vocational programs and work training, employment opportunities for efficient community participation and independent living,
2. Provide a flexible and individualized support system for children and youth with special needs in a regular class environment in schools nearest the students' home.
3. Implement a life-long curriculum to include early intervention and parent education, basic education and transition programs on vocational training or preparation for college, and
4. Make available an array educational programs and services: The Special Education Center built on "a school within a school concept" as the resource center for children and youth with special needs; inclusive education in regular schools, special and residential schools, homebound instruction, hospital instruction and community-based programs; alternative modes of service delivery to reach the advantaged children in far-flung towns, depressed areas and underserved barangays.

Historical Milestones of Special Education in the Philippines

1902- The interest to educate Filipino children with disabilities was expressed through Mr. Fred Atkinson, the General Superintendent of Education.

1907- Special Education was formally started in the country by establishing the Insular School for the Deaf and Blind in Manila.

1927- The government established the Welfareville Children's Village, a school for people with mental retardation in Mandaluyong.

1945- The National Orthopedic Hospital School for the Crippled Children and Youth is established.

1949- Quezon City Science High School was inaugurated for gifted students.

- 1950-** PAD opened a school for the children with hearing impairment.
- 1953-** The Elsie Gaches Village was established in Alabang to take care of the abandoned and orphaned children and youth with physical and mental handicaps.
- 1956-** Special classes for the deaf in regular class were implemented.
- 1957-** The Bureau of Public Schools of the Department of Education and Culture created the Special Education Section of the Special Subjects and Service Education.
- 1960-** Some private college and universities started to offer special education courses on graduate school curriculum.
- 1963-** With the approval of R.A. No. 3562, the training of DEC teacher scholars for blind children started at the Philippine Normal University.
- 1965-** Marked the start of training programs for school administrators on the supervision of special classes held at UP.
- 1969-** Classes for socially maladjusted children were organized at the Manila Youth Reception Center.
- 1970-** Training of teacher for Children with behavior problems started at the University of the Philippines.
- 1973-** The juvenile and domestic Relations Court of Manila established the Tahanan Special School for the socially maladjusted children and youth.
- 1975-** The Division of Manila City Schools implemented the Silahis Concept of Special Education in public elementary schools.
- 1979-** The Bureau of Elementary Education Special Education unit conducted a two-year nationwide survey if unidentified exceptional children who were in school.
- 1980-** The School for the Crippled Children at the Southern Island Hospital in Cebu City was organized.
- 1990-** The Philippine institute for the Deaf, an oral school for children with hearing impairment was established.
- 1992-** The summer training for teacher of the visually impaired started at the Philippine Normal University.
- 1993-** DECS issued Order No. 14 that directed regional officers to organize the Regional Special Education Council (RESC).
- 1995-** The summer training for teachers of the hearing impaired was held at Philippine Normal University.
- 1998-** DECS order No. 5 "Reclassification of Regular teacher and principal items to SPED teacher and special school's principal item".
- 1999-** DECS order no. 33 "Implementation of administrative order no. 101 directing the Department of Public Works and highways, the DECS and the CHED to provide architectural facilities or structural feature for disabled persons in all state college, universities and other buildings.
- 2000-** DECS Order No. 11, s. 2000 - Recognized Special Education (SPED) Centers in the Philippines.
- 2002-** An ongoing mobile teacher-training program by the Department of Education and the University of the Philippines trains regular and special education teachers on how to educate children with special needs.
- 2007-** Special Education Act of 2007 identifies ten groups of Children with Special Needs.
- 2009-** DepEd under its wing had 217 SPED Centers that cater to the needs of children with special abilities. The department issued Braille textbooks to help especially visually impaired children.

2010- Special Education Act of 2010, an act establishing at least one Special Education center for each school division and at least three Special Education centers in big school divisions for children with special needs, guidelines for government financial assistance and other incentives and support.

2012- DepEd has increased the funding for its Special Education program and is set to open new centers.

2013- DepEd organized a National Conference for SPED Teachers to sharpen their skills.

Legal Bases of SPED

Articles 356 and 259 of Commonwealth Act No. 3203

"the right of every child to live in an atmosphere conducive to his physical, moral and intellectual development" and the concomitant duty of the government "to promote the full growth of the faculties of every child."

Republic Act No. 3562: "An Act to Promote the Education of the Blind in the Philippines provided for the formal training of special education teachers of blind children at the Philippine Normal College, the rehabilitation of the Philippine National School for the Blind (PNSB) and the establishment of the Philippine Printing House of the Blind.

Republic Act No. 5250: "An Act Establishing a Ten-Year Teacher Training Program for Teachers of Special and Exceptional Children." Provided for the formal training of teachers for deaf, hard-of-hearing, speech handicapped, socially and emotionally disturbed, mentally retarded and mentally gifted and youth at the Philippine Normal College and the University of the Philippines

Section 8, Article XV of the 1973 Constitution of the Philippines: "A complete, adequate and integrated system of education relevant to the goals of national development."

Articles 3 and 74 of the Presidential Decree No. 603 of 1975

"The emotionally disturbed or socially maladjusted child shall be treated with sympathy and understanding and shall be given the education and care required by his particular condition."

"Thus, where needs warrant, there shall be at least special classes in every province, and if possible, special schools for the physically handicapped, the mentally retarded, the emotionally disturbed and the mentally gifted. The private sector shall be given all the necessary inducement and encouragement."

Presidential Decree No. 1509 of 1978: created the National Commission Concerning Disabled Persons (NCCDP) renamed as National Council for the Welfare of Disabled Persons (NCWDP).

Education Act of 1982 or Batas Pambansa Bilang 232: "The State shall promote the right of every individual to relevant quality education regardless of sex, age, breed, socioeconomical status, physical and mental condition, social and ethnic origin, political and other affiliations. The State shall therefore promote and maintain equality of access to education as well as enjoyment of the benefits of education by all its citizens."

Section 24 of BP 232: "Special Education Services": "the State further recognizes its responsibility to provide, within the context of the formal education system services to meet special needs of certain clientele. These specific types shall be guided by the basic policies of state embodied on General Provisions of this Act which include the education of persons who are physically, mentally, emotionally, socially, culturally different from the so-called 'normal' individuals that they require modification of school practices/services to develop to their maximum capacity."

Batas Pambansa Bilang 344: "An Act to Enhance the Mobility of Disabled Persons." Required cars, buildings, institutions, establishments, and public utilities to install facilities and other devices for persons with disabilities.

Republic Act No. 7277: Magna Carta for Persons with Disabilities: Persons with disability are part of Philippine society, and thus the State shall give full support to the improvement of their total well-being and their integration into the mainstream of society. They have the same rights as other people to take their proper place in society. They should be able to live freely and as independently as possible. This must be the concern of everyone the family, community and all government and non-government organizations. Rights of persons with disability must never be perceived as welfare services. Prohibitions on verbal, non-verbal ridicule and vilification against persons with disability shall always be observed at all times.

Republic Act No. 9442 Article XIV, Sections 1 and 2 of the 1987 Constitution of the Philippines: "The State shall protect and promote the right of all citizens to quality education at all levels and shall take appropriate steps to make such education accessible to all."

"The State shall provide adult citizens the disabled and out-of-school youth with training in civics, vocational efficiency and other skills."

Special Education in the Philippines is anchored on the following fundamental legal documents:

- An Act Providing for The Rehabilitation, Self-Development and Self-Reliance of Disabled Person and Their Integration into The Mainstream of Society and For Other Purposes
- An Act Amending Republic Act No. 7277, Otherwise known as the Magna Carta for Persons with Disability as Amended, and For Other Purposes' Granting Additional Privileges and Incentives and Prohibitions on Verbal, Non-Verbal Ridicule and Vilification Against Persons with Disability.

Special Education Division of the Bureau of Elementary Education

- Formulate policies, plans and programs;
- Develop standards of programs and services;
- Monitor and evaluate the efficiency of programs and services;
- Conduct in-service training programs to upgrade the competencies of special education administrators, teachers and ancillary personnel; and
- Establish and strengthen linkages and networks.

Prevalence of Children and Youth with Special Needs

Prevalence- refers to the total number of cases of a particular condition, those with exceptionality (giftedness and talent) and developmental disabilities and impairments.

Identifiable prevalence- refers to the cases that have come in contact with some systems.

True prevalence- assumes that there are a larger number of children and youth with special needs who are in school or in the community who have not been identified as such and are not in the special education programs of Department of Education.

The ultimate goal of special education in the public schools, according to the Special Education Division of DepEd, is the integration or mainstreaming of learners with special needs into the regular school system and eventually in the community, following the principles of The Salamanca Statement

on Principles, Policy and Practice in Special Education. Out of 84.4 million Filipinos, approximately 5.486 million (13%) are individuals with special needs. Around 4.8% are provided with appropriate educational services, but the 95.2% of those with exceptionalities are unserved. In 1995 to 1996 about 80,000 special needs children enrolled. 156,270 by school year 2004 to 2005: 77,152 were mentally gifted and 79,118 were children with disabilities:

- 40,260 learning disabilities
- 11,597 hearing impaired
- 2,670 visually impaired
- 12,456 intellectually disabled
- 5,112 behavior issues
- 760 orthopedically disabled
- 5,172 children with autism
- 912 speech defectives
- 142 chronically ill
- 32 children with cerebral palsy

Range of Special Education Programs and Services

Center- is a service delivery system which operates on the "school within a school" concept.

Special class or self- contained class- is the most popular type among the special education programs.

Integration and mainstreaming programs- have followed children and youth with disabilities to study in regular classes and learn side by side with their peers for the last forty years.

Partial mainstreaming- children who have moderate or severe forms of disabilities are mainstreamed in a regular class.

Full mainstreaming- children with disabilities are enrolled in regular classes and recite in all the subjects.

Special day school- serves one or more types of disabilities.

Residential school- provides both special education and dormitory services for its students.

Class Size

Exceptionality	One Grade Level	Multi-grade or Multi-Level
Gifted/Fast Learner	30-35	15-20
Mentally Retarded	8-15	8-10
Blind	7-10	5-6
Deaf	7-15	6-8
Behavior Problems	15-20	7-12
Orthopedically Handicapped	10-15	10-15
Speech defective	10-15	10-15
Learning Disabled	7-10	5-6
Multiple Handicapped	5-8	3-6

Difference between Special Education and Regular (General Education)

Kind Of Learners	Children with Special needs who necessitate individualized	Regular Learners
Number of Learners	For one-on-one: 1 For group: 3-4 For resource group : 5-10	Private School: 35-50 Public School: 50-60 or more
Emphasis	On the given intervention	On the curriculum and the mastery of skills established within the curriculum
Curriculum	Attention Skills (ADHD), Memory Skills (MR), Braille (VI), Sign Language (HI), Auditory and Visual Perception (LD), Anger Management (Emotional/ Behavioral Disturbance), Creative and Critical Thinking Skills (Giftedness), Functional Academics (Autism)	English, Reading, Math, Makabayan.....
Who dictates the curriculum	Individual needs of the child	School Systems (DECS)
Instruction	Intensive Systematic Instruction	Generalized Instruction
Received Instruction from?	SPED teachers, Occupational Therapist, Physical Therapist, Speech Pathologist	Regular Teachers (Subject Teachers)
Primary Function	Of SPED Teacher: To meet the goals and objectives established in the IEP	Of Regular Teacher: To teach the curriculum
Type of Classroom	SPED Classroom, Self-contained Room, Resource Room, Mainstream Class	Regular Classroom

What Is Inclusive Education?

- inclusive education is provided by Lipsky & Gartner (1996, 1999), who described it as students with disabilities having full membership in age-appropriate classes in their neighborhood schools, with appropriate supplementary aids and support service.
- to Antia et al. (2002), inclusion denotes a student with a disability unconditionally belonging to and having full membership of a regular classroom in a regular school and its community.
- inclusive education is a process whereby the school systems, strategic plans, and policies adapt and change to include teaching strategies for a wider more diverse range of children

Inclusive Education in the Philippines POLICY:

- Provides "all children and youth with special needs must receive an appropriate education and everything within the system and they should get education along with other children regardless of human differences". This requires fundamental changes in philosophy, curriculum, teaching strategies and structural organization and above all changes in human attitudes and behaviors of all those involved in the education of the children with special needs

Special Education

- It is an individually planned, systematically implemented and carefully evaluated instruction to help exceptional children achieve the greatest possible personal self-sufficiency and success in present and future environments.
- It can be differentiated from regular education by its curriculum- some children need intensive, systematic instruction to learn skills that normally developing children acquire naturally.
- It provides learning opportunities that are not provided in standard or regular school curricula or by regular school services.

WHERE

- **Schools** – special and regular schools; therapy or early intervention centers; home; vocational centers; community

HOW

- Use of varied teaching methods and strategies
- Special educators should be skilled in the procedures for systematically designing, implementing and evaluating instruction.

Special Education as Intervention

- Intervention is a general name for all efforts in behalf of individuals with disabilities.
- The goal of SPED is to eliminate or at least reduced the obstacles that might keep a child or adult with disabilities from full and active participation in school and society.

Preventive Techniques:

- Social and educational programs to stimulate infants and very young children to acquire skills that most children learn normally and without help.
- Remedial Programs
- Compensatory Efforts (Aims to give an individual some kind of asset to be able to use tools and techniques for meeting learning needs.

Exceptional Children

- Whose special needs or abilities necessitate an individualized program of education.
- Refers to individuals who differ from societal or community standards of normalcy.
- Differ from the norm either below or above to such an extent that an individualized program of special education is required to meet their needs.
- Defines the exceptional child as one who deviates from the average or normal child.
- It is an inclusive term that refers to individuals with learning or behavior problems, individuals with physical and sensory disabilities and those who are intellectually gifted.

Classification of Exceptionalities

- ✓ Autism
- ✓ Deafness
- ✓ Deaf-Blindness
- ✓ Hearing Impairment
- ✓ Mental Retardation
- ✓ Multiple Disabilities
- ✓ Orthopedic Impairment
- ✓ Health Impairment
- ✓ Serious Emotional disturbance
- ✓ Specific Learning Disability
- ✓ Speech or Language Impairment
- ✓ Traumatic Brain Injury
- ✓ Visual Impairment
- ✓ Gifted and Talented

Disability – refers to an individual to do something in a certain way; an incapacity to perform as other individuals due to impairments in sensory, physical, cognitive and other areas of functioning.

- Refers to the loss or reduced function of a particular body part or organ.
- Synonymous to impairment.
- It limits the ability to perform certain tasks.
- A disabled person is not handicapped unless the physical disability leads to educational, personal, social, vocational and other problems.

Handicap – refers to the problems that an individual with a disability encounters as he or she attempts to function or interact in his or her environment; this term is used when explaining the consequences or impact imposed on the individual by his or her disability.

Goal of Special Education (From Article I, Section 5 of Child and Youth Welfare Code PD No. 603)

1. Integration or mainstreaming of learners with special needs into the regular school system and eventually in the community.
2. To meet the individual educational needs of the learners and, to the extent possible, prepare them for going to a more regular classroom setting.
3. Special Educational programs must focus on helping the learners develop academic skills, self-help skills, social proficiency, a positive attitude and self-confidence.

Individuals with Disabilities Education Act (IDEA)

- Is a United States federal law that governs how states and public agencies provide early intervention, special education, and related services to children with disabilities. It addresses the educational needs of children with disabilities from age 3 to age 18 or 21 in cases that involve 14 specified categories of disability.
- In defining the purpose of special education, IDEA 2004 clarifies Congress' intended outcome for each child with a disability: students must be provided a Free Appropriate

Public Education (FAPE) that prepares them for further education, employment and independent living.

Under IDEA 2004:

- Special education and related services should be designed to meet the unique learning needs of eligible children with disabilities, preschool through age 21.
- Students with disabilities should be prepared for further education, employment and independent living.

Individualized Education Program (IEP)

- An IEP defines the individualized objectives of a child who has been found with a disability, as defined by federal regulations. The IEP is intended to help children reach educational goals more easily than they otherwise would. In all cases the IEP must be tailored to the individual student's needs as
- identified by the IEP evaluation process, and must especially help teachers and related service providers
- (such as paraprofessional educators) understand the student's disability and how the disability affects the learning process.
- The IEP describes how the student learns, how the student best demonstrates that learning and what teachers and service providers will do to help the student learn more effectively. Developing an IEP requires assessing students in all areas related to the known disabilities, simultaneously considering ability to access the general curriculum, considering how the disability affects the student's learning, forming goals and objectives that correspond to the needs of the student, and choosing a placement in the least restrictive environment possible for the student.
- An IEP is meant to ensure that students receive an appropriate placement, not "only" special education classrooms or special schools. It is meant to give the student a chance to participate in "normal" school culture and academics as much as is possible for that individual student. In this way, the student is able to have specialized assistance only when such assistance is absolutely necessary, and otherwise maintains the freedom to interact with and participate in the activities of his or her more general school peers.

Dizon's Concept of Normalization

a. Integration

- consist of placing the special learner in the regular class in one or more subjects, so that he can interact with regular children with the SPED class as his station; may include tutorials and involve ancillary or auxiliary services.

b. Mainstreaming

- in the context of education, is the practice of educating students with special needs in regular classes during specific time periods based on their skills.[1] This means regular education classes are combined with special education classes. Schools that practice mainstreaming believe that

students with special needs who cannot function in a regular classroom to a certain extent "belong" to the special education environment.

Benefits of Mainstreaming to Students with Disabilities

1. Higher academic achievement:
2. Higher self-esteem
3. Better social skills

c. Inclusion

- Wherein the special learner is placed in the regular class based on age; regardless of the background, disability and degree of severity. This is no pull out and no shadow teaching.

d. Segregation

- A segregated student attends no classes with non-disabled students. He or she might attend a special school that only enrolls other students with disabilities, or might be placed in a dedicated, self-contained classroom in a school that also enrolls general education students.
- Some students may be confined to a hospital due to a medical condition and are thus eligible for tutoring services provided by a school district. Less common alternatives include homeschooling and, particularly in developing countries, exclusion from education

Application:

1. What Is the status of special education programs in the Philippines? How does DepEd sustain it SPED programs?
2. What can persons with disabilities do despite their handicaps?
3. Define and explain the following terms:
 1. Exceptional children
 2. SPED
 3. Disability
 4. Handicap
 5. At risk

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output.

Criteria	5	3	2	1
Focus and Details	There is one clear, focused topic. Main idea is clear and well supported by detailed and accurate information.	There is one clear, well focused topic. Main idea is clear but not well supported by detailed information.	Main idea is somewhat clear.	The topic and main idea are not clear.
Organization	The introduction states the main topic and provides an overview of the essay. Information is relevant and presented in a logical order. The conclusion is good.	The introduction states the main topic and provides an overview of the essay. A conclusion is included.	The introduction states the main topic. A conclusion is included.	There is no clear introduction, structure, or conclusion.

Word Choice	It uses vivid words and phrases. The choice and placement of words seems accurate, natural, and not forced.	It uses vivid words and phrases. The choice and placement of words is inaccurate at times and/or seems overdone.	It uses words that communicate clearly, but the writing lacks variety.	It uses a limited vocabulary. Jargon or clichés may be present and detract from the meaning.
Sentence structure, grammar and mechanics	All sentences are well constructed and have varied structure and length. There are no errors in grammar, mechanics, and/or spelling.	Most sentences are well constructed and have varied structure and length. There are few errors in grammar, mechanics, and/or spelling, but they do not interfere with understanding.	Most sentences are well constructed, but they have a similar structure and/or length. There are several errors in grammar, mechanics, and/or spelling that interfere with understanding.	Sentence's sound awkward, are distractingly repetitive, or are difficult to understand. There are numerous errors in grammar, mechanics, and/or spelling that interfere with understanding.

Adapted from: <http://www.readwritethink.org/files/resources/printouts/Essay%20Rubric.pdf>

Summary of the Lesson:

The ultimate goal of special education shall be the integration or mainstreaming of learners with special needs into the regular school system and eventually in the community.

Special education shall aim to develop the maximum potential of the child with special needs to enable him to become self-reliant and shall be geared towards providing him with the opportunities for a full and happy life.

The specific objectives of special education shall be the development and maximization of learning competencies, as well as the inculcation of values to make the learners with special needs a useful and effective member of society.

Exceptional children and youth like all other pupils in regular classes are individuals with their unique traits and characteristics. Some learn slower than the average pupils like those with mental retardation. However, despite of different disabilities and exceptionalities, they are capable of showing appreciation, gratitude, love and friendship. To address this, SPECIAL EDUCATION DIVISION OF THE BUREAU OF ELEMENTARY EDUCATION formulate policies, plans and program, develop standards of programs and services, monitor and evaluate the efficiency of programs and services, conduct in-service training programs to upgrade the competencies of special education administrators, teachers and ancillary personnel; and establish and strengthen linkages and networks.

What is Special Education?

- a classroom or private instruction involving techniques, exercises, and subject matter designed for students whose learning needs cannot be met by a standard school curriculum.
- an education modified for those with disabilities or exceptional needs as handicapped people or gifted children.

Specially Designed Instruction means that a student's progress in learning is monitored frequently, often several times per week.

Specially Designed Instruction COMPONENTS:

1. Intensive instruction

- translates into more teacher instructional time and more opportunities for students to respond to the instruction and more time to practice and review what they have learned.

2. Relentless instruction

- involves repeating on what is to be learned more often than is typically done with non-disabled students.

3. Structured instruction

- refers to teachers being more directive, instituting more directive, instituting more explicit rules, and providing more frequent consequences for appropriate or inappropriate behavior.

4. Pace of the instruction.

- is tailored more to the needs of students and is often slower, with teachers waiting for a longer period for a response after querying the student.

5. Instruction in small groups

- facilitates the intensity, relentlessness, structured nature, and individualized pace of instruction

Individualized Education Program (IEP)

- describes the special education and related services specifically designed to meet the unique educational needs of a student with a disability;
- the guiding document for a student's educational program; and
- It includes all of the goals, objectives, present levels of performance and related services that are recommended for the student.

Assessment:

1. Interview one SPED student within your community (Virtual/Thru Message or chat/ By any safe means)
1. What is his/her exceptionality?
2. How does their schoolwork during this time?
3. What are the difficulties encountered?
4. How do they cope up?
5. Provide a picture/proof of interview (screenshot will do if interview was done thru chat/text)

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output.

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Adapted from: <http://www.readwritethink.org/files/resources/printouts/Essay%20Rubric.pdf>

II. Fill in the matrix below to illustrate salient features, similarities, and differences among types of SPED services.

Types of SPED Services	Salient features, similarities, and differences
1.	
2.	
3.	
4.	
5.	

III. Fill in the matrix with definition, learning and behavioral characteristics or categories of children with special needs.

Category of CSN	Definition, Types, Characteristics

Enrichment Activity:

1. What is the importance of legislation in the development and sustenance of SPED programs?
2. What is the impact of the number of Filipino with special needs to you?
3. Make a reaction paper out of this video: <https://www.youtube.com/watch?v=j5lh8sb9mhg>.

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output

Criteria	5	3	2	1
Focus and Details	There is one clear, focused topic. Main idea is clear and well supported by detailed and accurate information.	There is one clear, well focused topic. Main idea is clear but not well supported by detailed information.	Main idea is somewhat clear.	The topic and main idea are not clear.
Organization	The introduction states the main topic and provides an overview of the essay. Information is relevant and presented in a logical order. The conclusion is good.	The introduction states the main topic and provides an overview of the essay. A conclusion is included.	The introduction states the main topic. A conclusion is included.	There is no clear introduction, structure, or conclusion.
Word Choice	It uses vivid words and phrases. The choice and placement of words seems accurate, natural, and not forced.	It uses vivid words and phrases. The choice and placement of words is inaccurate at times and/or seems overdone.	It uses words that communicate clearly, but the writing lacks variety.	It uses a limited vocabulary. Jargon or clichés may be present and detract from the meaning.
Sentence structure, grammar and mechanics	All sentences are well constructed and have varied structure and length. There are no errors in grammar, mechanics, and/or spelling.	Most sentences are well constructed and have varied structure and length. There are few errors in grammar, mechanics, and/or spelling, but they do not interfere with understanding.	Most sentences are well constructed, but they have a similar structure and/or length. There are several errors in grammar, mechanics, and/or spelling that interfere with understanding.	Sentence's sound awkward, are distractingly repetitive, or are difficult to understand. There are numerous errors in grammar, mechanics, and/or spelling that interfere with understanding.

Adapted from: <http://www.readwritethink.org/files/resources/printouts/Essay%20Rubric.pdf>

Suggested Links (Optional):

- <https://www.scribd.com/document/394852108/Historical-Perspectives-of-Sped>
- <https://eedncr.wordpress.com/about-us/special-education/>
- <https://www.slideshare.net/MariaMarthaManetteMadrid/vision-policy-goal-and-objectives-of-special-education-in-the-philippines>
- <http://docshare01.docshare.tips/files/23840/238407438.pdf>
- <http://docshare01.docshare.tips/files/23840/238407438.pdf>
- <https://www.slideshare.net/gingjandi/chapter-2-special-program-and-services-for-special-children>

References/Attributions:

- Inciong, Teresita G., Quijano, Yolanda S. and Capulong, Yolanda T. Introduction to Special Education. Manila, Philippines: Rex Bookstore, 2007.

Lesson 2: Students with Mental Retardation**Lesson Objectives:**

1. Define mental retardation and explain the four factors and five assumptions in the definition;
2. Enumerate and discuss the classification of mental retardation;
3. Identify and explain the causes of mental retardation during the phases of prenatal development, the birth process, infancy, and early childhood;
4. Name and describe the assessment procedures to screen and assess children with mental retardation; and
5. Enumerate and describe the educational approaches in teaching children and youth with mental retardation.

Getting Started (Optional):

"There is no one who cannot find a place for himself in our kind of world. Each one of us has some unique capacity for realization. Every person is valuable in his or her own existence – for himself."

George H. Bender

1. What can you say about this quote?
2. Could you cite your own characteristic/s that makes you unique? Why?

Discussion:

Mental retardation refers to substantial limitations in present functioning. It is characterized by significantly sub-average intellectual functioning, existing concurrently with related limitations in two or more of the following adaptive skills areas: communication, self-care, home living, social skills, community use, self-direction, health and safety, functional academics, leisure, and work. Mental retardation manifests before age 18.

There are four criteria in the definition:

- **Substantial limitations in present functioning** means that the person has difficulty in performing everyday activities related to taking care of one's self, doing ordinary tasks at home and work related to the other adaptive skills areas.
- **Significantly sub-average intellectual functioning** means that the person has significantly below average intelligence. Intellectual functioning is a broad summation of cognitive abilities such as the capacity to learn, solve problems, accumulate knowledge, and adapt to new situations.
- **Limitations.** Persons with mental retardation fail to meet the standards of personal independence and social responsibility expected of their chronological age and cultural group.
- **Related limitations in the adaptive skills areas** means that the person has difficulty in performing the following tasks: (Beirne-Smith, 2002)
 1. **Communication** means the ability to understand and communicate information by speaking and writing through symbols, sign language and non-symbolic behavior like facial expressions, touch, or gestures.
 2. **Self-care** means the ability to take care of one's needs in hygiene, grooming, dressing, eating, toileting.
 3. **Home living** means the ability to function in the home, housekeeping, clothing care, property maintenance, cooking, shopping, home safety, daily scheduling of work.
 4. **Community use** or travel in the community, shopping, obtaining services.
 5. **Social skills** in initiating and terminating interactions, conversations, responding to social cues, recognizing feelings, regulating own behavior, assisting others, fostering friendship.
 6. **Self-direction** in making choices, following schedule, completing required tasks, seeking assistance, and resolving problems.
 7. **Health and safety** such as maintaining own health, identify and preventing illness, first aid, sexuality, physical fitness, and basic safety.
 8. **Functional academics** or learning the basic skills taught in school.
 9. **Leisure** such as recreational activities that are appropriate to the age of the person.
 10. **Work or employment**, appropriate to one's age.

Classification of Mental Retardation

The criteria in the AAMR definition are very extensive, thus, a system of sub-categories or levels of mental retardation was developed. Traditionally, sub-categories have been based on IQ ranges. In the previous AAMR classification system, there are four levels that are still widely used today:

1. mild MR with IQ scores from 55 to 70
2. moderate MR with IQ scores from 40 to 54
3. severe MR with IQ scores from 25 to 39, and
4. profound MR with IQ scores below 25

Causes of Mental Retardation

The AAMR classifies the causes or etiological factors based on time of onset, categorized as prenatal or biological (occurring before birth), perinatal (occurring during birth), and postnatal and environmental (occurring shortly after birth).

I. Prenatal causes includes:

- **Down syndrome** is caused by chromosomal abnormality, the most common is trisomy 21 in which the 21st set of chromosomes is a triplet rather than a pair. Trisomy 21 often results in moderate level of mental retardation, although some individuals function in the mild or severe ranges.
- **Klinefelter syndrome**, males receive an extra X chromosome. Sterility, underdevelopment of male sex organs, acquisition of female secondary sex characteristics is common.
- **Fragile X syndrome** a triplet or repeat mutation on the X chromosome interferes with the production of FMR-1 protein which is essential for normal brain functioning. It is characterized by social anxiety, avoiding eye contact, tactile defensiveness, turning the body away during face-to-face interactions and stylized, ritualistic forms of greetings.
- **William syndrome** is caused by the deletion of a portion of the seventh chromosome. The characteristics are elfin or dwarf-like facial features; physical features and manner of expression exudes cheerfulness and happiness; "overly friendly", lack of reserve toward strangers, often have uneven profiles of skills, with strengths in vocabulary and storytelling skills and weaknesses in visual-spatial skills; often hyperactive, may have difficulty staying on task and low tolerance for frustrations or teasing.
- **Prader-Willi syndrome** is a syndrome disorder caused by the deletion of a portion of chromosome 15. It is associated with mild retardation and learning disabilities. The common behavioral problems are impulsivity, aggressiveness, temper tantrums, obsessive-compulsive behavior, and some forms of injurious behavior.
- **Phenylketonuria (PKU)**. PKU is a genetically inherited condition in which a child is born without an important enzyme needed to break down an amino acid called phenylalanine found in dairy products and other protein-rich foods. Failure to break down this amino acid causes brain damage that often results in aggressiveness, hyperacidity, and severe mental retardation.

II. Perinatal causes includes:

- **Intrauterine disorders** include maternal anemia, premature delivery, abnormal presentation, umbilical cord accidents and multiple gestation in the case of twins, triplet, quadruplets, and other types of multiple births.
- **Neonatal disorders** such as intracranial hemorrhage, neonatal seizures, respiratory disorders, meningitis, encephalitis, head trauma at birth.

III. Postnatal causes include:

- Head injuries such as cerebral concussion, contusion, or laceration
- Infections such as encephalitis, meningitis, malaria, German measles, rubella
- Demyelinating disorders such as post infectious disorders, post immunization disorders
- Degenerative disorders such as Rett syndrome, Huntington disease, Parkinson's disease
- Seizure disorders such as epilepsy, toxic-metabolic disorders such as Reye's syndrome, lead, or mercury poisoning
- Malnutrition especially lack of proteins and calories
- Environmental deprivation such as psychosocial disadvantage, child abuse and neglect, chronic social/sensory deprivation; and

- Hypoconnection syndrome.

Learning Behavior and Characteristics

I. Deficits in Cognitive Functioning

Sub-Average Intellectual Skills. The first defining characteristic of persons with mental retardation is below average mental ability as measured by standardized tests.

- *Low Academic Achievement.* Persons with mental retardation are likely to be slower in reaching levels of academic achievement equal to their peers.
- *Difficulty in Attending to Tasks.* The attention of these children tends to be distracted by irrelevant stimuli rather than those that pertain to the lesson.

II. Deficits in Memory

- *Difficulty with the generalization of skills.* The inability to generalize is related to the inability to think abstractly.
- *Low motivation.* Some students show lack of interest in learning their lessons. To avoid failure, the person tends to set very low expectations for oneself.

III. Deficits in Adaptive Behavior

- *Self-care and daily living skills.* They are often taught basic self-care skills deliberately which normal individuals learn by absorption and imitation.
- *Social development.* Limited cognitive processing skills, poor language development, and unusual or inappropriate behaviors can seriously impede interaction with others. Thus, making friends and sustaining personal relationships are difficult for persons with mental retardation.
- *Behavioral excesses and challenging behavior.* Students with mental retardation are more prone to inappropriate behavior such as difficulties in accepting criticism, limited self-control, as well as behavior problems like aggression or self-injury.
- *Psychological Characteristics.* As in the case of speech and language problems, mentally retarded persons have slower psychological development and are likely to have some forms of associated physical problems.
- *Positive Characteristics.* Persons with mental retardation have their unique characteristics like friendliness and kindness. They can be fun to be with and they can get along well with others

Assessment Procedures

Models of Assessment (Richey and Wheeler, 2000)

1. *Traditional Assessment.* The parents fill in a pre-referral form about the family history and the developmental history of the child. Then the child and parents are referred to a team of clinical practitioners for thorough evaluation of the child's intellectual, socio-emotional, and physical development, health condition, and other significant information.
2. *Team-Based Assessment Approaches.* It is a multidisciplinary, interdisciplinary, and transdisciplinary in nature. In multidisciplinary assessment, individual team members independently assess the child and report results without consulting or integrating their findings with one another. While interdisciplinary assessment, the members conduct an independent

assessment and evaluation individually the findings are integrated together with the recommendations. Meanwhile, transdisciplinary assessment allows other team members as facilitators during the assessment process. A natural extension of this approach is the involvement of the family in the decision-making process.

3. *Activity-Based Assessment*. This activity-based model of assessment is better than other models because of parental involvement as well as the development of meaningful, child-centered, positive behavioral supports and activity-based interventions. The assessment materials have a curriculum and evaluation components and do not require specialized materials or test kits.

Educational Programs

Models of Early Intervention

1. *Home-Based Instruction Program*

The goal is to provide a continuous program of instruction both in school and at home for a more effective management of the handicapping condition. The key person are the biological or surrogate parents who perform their primary role as caregivers.

2. *Head Start Program*

The program operates on the principle of early intervention as a preventive measure against behavior problems among young children that may lead ultimately to juvenile delinquency. The participants are children and siblings of youth offenders, slum dwellers, street children and others pre-school age.

3. *Community-Based Rehabilitation (CBR) Services*

The World Health Organization (WHO, 1984) defines community-based rehabilitation as measures taken at the community level that use and build on the resources of the community to assist in the rehabilitation of those who need assistance including the disabled and handicapped persons, their families, and their community.

4. *Urban Basic Service Program*

An early intervention scheme based on the principles of home-based instruction was adopted by the Urban Basic Service Program as its education component. The program also utilized the Filipino adaptation of the Portage Guide to Early Intervention.

Educational Approaches

The Curriculum

The *Cognitive Curriculum for Young Children (CCYC)* is a major curriculum effort that is based on Piaget's Theory of cognitive development, Vygotsky's zone of proximal development and Feuerstein's concept of mediated learning. The CCYC builds its instructional program around the child's deficits in cognition where mediated learning is applied. Estimate of a child's maximum learning potential are derived from his zone of proximal development that is determined by comparing the child's actual level of performance to his performance the direct teacher's observation.

The *Instrumental Enrichment* program wherein the child is trained to develop a sense of intentionality and a feeling of competence because of structured mediated learning environments.

The *Montessori Method* aims to develop the child's sense of self mastery, mastery of the environment and independence by focusing on his or her perceptual and conceptual development as well as in the acquisition of skills in self-care and daily living activities.

The *Ypsilante Perry Preschool Project* were derived from Piaget's cognitive development theory. The cognitively oriented curriculum is used in teaching disadvantaged children with mild retardation who are three to four years old.

The *Portage Project* uses the precision teaching model to deliver a home-based curriculum in language, self-help skills, cognition, motor skills and socialization. The parents are trained to teach their children using behavior modification procedures.

The *Carolina's Abecedarian Project* includes parent training, social work services, nutritional supplement, medical care and transportation. Its curriculum is designed around the interaction of consumer opinions or the goals that parents have for their children, Piaget's developmental theory, developmental facts, adaptive sets and high risked indicators.

Application:

Answer the following questions on how you have learned about mental retardation.

1. What makes mental retardation a complex, rather than a simple developmental disability?
2. Enumerate and describe the learning and behavior characteristics of a person with mental retardation?

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output.

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Focus and Details	There is one clear, focused topic. Main idea is clear and well supported by detailed and accurate information.	There is one clear, well focused topic. Main idea is clear but not well supported by detailed information.	Main idea is somewhat clear.	The topic and main idea are not clear.
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Sentence structure, grammar and mechanics	All sentences are well constructed and have varied structure and length. There are no errors in grammar, mechanics, and/or spelling.	Most sentences are well constructed and have varied structure and length. There are few errors in grammar, mechanics, and/or spelling, but they do not interfere with understanding.	Most sentences are well constructed, but they have a similar structure and/or length. There are several errors in grammar, mechanics, and/or spelling that interfere with understanding	Sentences sound awkward, are distractingly repetitive, or are difficult to understand. There are numerous errors in grammar, mechanics, and/or spelling that interfere with understanding.
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Summary of the Lesson:

Mental retardation refers to substantial limitations in present functioning. It is characterized by significantly sub-average intellectual functioning, existing concurrently with related limitations in two or more of the following adaptive skills areas: communication, self-care, home living, social skills, community use, self-direction, health and safety, functional academics, leisure, and work. Mental retardation manifests before age 18.

The AAMR classifies the causes or etiological factors based on time of onset, categorized as prenatal or biological (occurring before birth), perinatal (occurring during birth), and postnatal and environmental (occurring shortly after birth). The specific *biological causes* are known for about two-thirds of individuals with the more severe forms that include the moderate, severe, and profound types. The *environmental causes* are traced to a psychological disadvantage which is a combination of a poor social and cultural environments early in the child's life.

The learning and behavior characteristics of the child with mental retardation involves Deficits in Cognitive Functioning, Deficits in Memory, and Deficits in Adaptive Behavior

The assessment used to assess the child with mental retardation are Traditional Assessment, Team-Based Assessment Approaches, and Activity-Based Assessment.

The educational programs that were used are the following Home-Based Instruction Program, Head Start Program, Community-Based Rehabilitation (CBR) Services, and Urban Basic Service Program.

The following are the curriculum used for the children with mental retardation. These are *Cognitive Curriculum for Young Children (CCYC)*, *Instrumental Enrichment* program, the *Montessori Method*, *Ypsilante Perry Preschool Project*, the *Portage Project*, and the *Carolina's Abecedarian Project*.

Assessment:

Multiple Choice: Choose the letter of the correct answer. Write your answers on the space provided.

- _____ 1) Which of the following is an example of an intellectual disability?
A. Dyslexia
B. ADHD
C. Mental Retardation
D. Autistic spectrum disorders
- _____ 2) Reading Disorder is a developmental disorder and is characterized by reading achievement (e.g. accuracy, speed and comprehension) being significantly below standards expected for which of the following
A. Chronological age
C. Schooling experience

- B. IQ
D. All of the above
- ____ 3) In DSM-IV-TR intellectual disabilities are divided into several degrees of severity, depending primarily on the range of IQ score provided by the sufferer. One of these is Mild Mental Retardation, corresponding to an IQ score between:
A. 60 – 65 to 80
B. 40 – 55 to 60
C. 50 – 55 to 70
D. 70 – 75 to 90
- ____ 4) In DSM-IV-TR intellectual disabilities are divided into several degrees of severity, depending primarily on the range of IQ score provided by the sufferer. One of these is Moderate Mental Retardation, corresponding to an IQ score between:
A. 60 – 65 to 70 - 75
B. 35 – 40 to 50 - 55
C. 80 – 85 to 90 - 95
D. 20 – 25 to 30 – 35
- ____ 5) In DSM-IV-TR intellectual disabilities are divided into several degrees of severity, depending primarily on the range of IQ score provided by the sufferer. One of these is Severe Mental Retardation, corresponding to an IQ score between:
A. 5 – 10 to 15 – 20
B. 30 – 35 to 45 – 50
C. 10 – 15 to 20 – 25
D. 20 – 25 to 35 - 40
- ____ 6) In DSM-IV-TR intellectual disabilities are divided into several degrees of severity, depending primarily on the range of IQ score provided by the sufferer. One of these is Profound Mental Retardation, corresponding to an IQ score between:
A. 20 – 25
B. 25 – 30
C. 15 – 20
D. 10 – 15
- ____ 7) Some of the common behavioral problems of the child with mental retardation are impulsivity, aggressiveness, temper tantrums, obsessive-compulsive behavior, delayed motor skills, etc.
A. Down syndrome
B. William syndrome
C. Prader-Willi syndrome
D. Fragile X syndrome
- ____ 8) This syndrome is characterized by social anxiety, avoiding eye contact, tactile defensiveness, turning the body away during face-to-face interactions and stylized, ritualistic forms of greetings.
A. Down syndrome
B. William syndrome
C. Prader-Willi syndrome
D. Fragile X syndrome
- ____ 9) It is a form of assessment where every individual team member independently assesses the child and report results without consulting or integrating their findings with one another.
A. Traditional Assessment
B. Multidisciplinary Assessment
C. Interdisciplinary Assessment
D. Transdisciplinary Assessment
- ____ 10) It is an educational approach which aims to develop the child's sense of self mastery, mastery of the environment and independence by focusing on his or her perceptual and conceptual development and in the acquisition of skills in self-care and daily living activities.
A. Montessori Method
B. Carolina's Abecedarian Project
C. Instrumental Enrichment program
D. Ypsilante Perry Preschool Project

Enrichment Activity:

How do we deal students with mental retardation? If possible, interview one physician that is expert in the field?

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output.

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Suggested Links (Optional):

- <https://mn.gov/mnddc/parallels2/pdf/70s/72/72-COF-NIO.pdf>

References/Attributions:

- Inciong, Teresita G., Quijano, Yolanda S. and Capulong, Yolanda T. Introduction to Special Education. Manila, Philippines: Rex Bookstore, 2007.

Lesson 3: Students with Learning Disabilities

Lesson Objectives:

1. Define the term learning disabilities; explain the criteria in determining the presence of learning disabilities;

2. Define Attention Deficit/Hyperactivity Disorder; explain the syndromes in the areas of inattention, hyperactivity and impulsivity;
3. Explain the causes or etiology of learning disabilities;
4. Discuss the assessment procedures in identifying students with learning disabilities; and
5. Enumerate and describe the special education programs for students with learning disabilities.

Getting Started (Optional):

1. What is mental ability?
2. What is learning disability?

Discussion

Learning Disabilities refers to a heterogeneous group of disorders manifested by significant difficulties in the acquisition and use of listening, speaking, reading, writing, reasoning, or mathematical abilities. These disorders are intrinsic to the individual and presumed to be due to central nervous system dysfunction. Problems in self-regulatory behavior, social perception and social interaction may exist in learning disabilities but do not themselves constitute a learning disability.

The phenomenon is explained by the factor called *IQ achievement discrepancy*. The child does not learn knowledge and skills in accordance with his or her potential to learn as measured by a standardized mental ability test. While his or her mental ability is appropriate to his or her chronological age, the child's performance in school shows that achievement is two or more years below the grade level.

Learning disabilities is *intrinsic* to the individual. This means that the causes of learning disabilities are organic, biological, genetic, or environmental. The organic and biological factors are traced to the central nervous system, particularly the brain. In most cases, the cause of learning disabilities remains a mystery.

Three Criteria in Determining the Presence of Learning Disabilities

1. *Severe discrepancy between the child's potential and actual achievement*
2. *Exclusion or absence of mental retardation, sensory impairment, and other disabilities*
3. *Need for special education services*

Learning and Behavior Characteristic of Children with Learning Disabilities

1. *Reading* poses the most difficult among all the subjects in the curriculum. The facets of communication are listening, speaking, reading, and writing. **Dyslexia** refers to a disturbance in the ability to learn in general and the ability to read.
2. *Written language* poses severe problems in one or more of the following areas: handwriting, spelling, composition, and writing which is illegible and slow.
3. *Spoken language* poses problems on the mechanical uses of language in syntax or grammar, semantics or word meanings and phonology or the breakdown of words into their component sounds and blending individual sounds to compose words.
4. *Pragmatics or social uses of language* poses problems on the ability to carry on a conversation.

5. *Mathematics problems* are recognized as second to deficiencies in reading, language, and spelling.
6. *These children tend to fail and be retained in a grade level.* The level of academic achievement tends to decrease progressively as the grade level increases.
7. *Behavior* the common behavior problems are inattention, impulsivity, and hyperactivity.
8. *Social acceptance* is low, but some can be popular.

Perceptual, Perceptual-Motor, and General Coordination Problems

1. Children with learning disabilities exhibit *visual and/or auditory perceptual disabilities*. The problem is lack of acuity or sharpness in vision or audition in responding to visual and auditory perceptual stimulation.
2. They have *difficulty with physical activities* that involve gross and fine motor skills. Thus, they tend to drop things as though they are “all thumbs” or have two left feet.
3. They have *problems with attention and hyperactivity*.

Memory, Cognitive, and Metacognitive Problems

Problems in memory, cognitive, and metacognitive areas are related. If there is a problem in memory then there are also problems in understanding or cognition. The problems show in difficulties in remembering assignments and appointments. Though smart, the students forget the lessons easily and show deficits in memory because they do not use memory strategies like rehearsal, categorizing and use of mnemonics.

Attention deficits. Selective attention or the inability to focus on the relevant details of the lesson is the first requirement for learning to take place. The deficit in attention results to inefficient learning or no learning at all.

Poor memory. Poor ability to store and retrieve information or previous learning is very evident among children with learning disabilities. They find difficulty in remembering mathematics facts, spelling words, vocabulary meaning, to organize information for recall. Common strategies for storing and recalling information such as verbal or written rehearsal, coding, or associating a new item with a concept already in memory, imagery and mnemonics are either absent, immature, or inefficiently used.

Problems in Social Competence

Problems in relating with other people is not a characteristic of persons with learning disabilities. Rather, it is an outcome of the different social climates created by people in school, at home, the community, and other places with whom persons with learning disabilities interact.

Causes of Learning Disabilities

The causes of learning disabilities are attributed to genetic and environmental factors. The genetic factors refer to the characteristics that are inherited through the genes, chromosomes, and deoxyribonucleic acid or DNA.

The Genetic Factors in Learning Disabilities

Learning disabilities constitutes a heterogeneous set of conditions with no single syndrome nor a single cause or etiology. Research findings cite two genetic causes of learning disabilities. These are brain damage and biochemical imbalance.

1. Brain Damage

An increasing consensus attributes learning disabilities to neurological dysfunction or central nervous system pathology. The models on the causes of learning disabilities state that the condition can be:

- a. *Hereditary*, indicating the presence in the genetic makeup of certain inherited diseases or disorders that damage the brain.
- b. *Innate*, resulting from biological influences during the period of conception or pregnancy.
- c. *Congenital*, or constitutional, indicating that biological influences may have originated during the process of gestation or development in the prenatal (before birth), perinatal (during birth), or postnatal (after birth) periods of development.

Neurologists and other professionals use advance technology to assess brain activities more accurately. Some of the new procedures are:

- a. *Electroencephalogram or EEG*. This is a graphic measure and recording of the brain's electrical impulses. The EEG is a digitally computerized recording and analysis of the brain waves.
- b. *Computerized Tomographic Scans or CT*. This is neuroimaging technique whereby X-rays of the brain are compiled by a computer to produce an overall picture of the brain.
- c. *Magnetic Resonance Imaging or MRI*. This neuroimaging technique whereby radio waves are used to produce cross-sectional images of the brain.

2. Biochemical Imbalance

A study in 1975 (Feingold) suggested that artificial food colors, flavors, preservatives, salicylates, and megavitamins in many of the foods that children eat can cause learning disabilities and hyperactivity. But related studies concluded that very little pieces of evidence supported the statement that biochemistry may affect a child's behavior and learning in the classroom, no scientific evidence exists today to reveal the nature or extent of that influence.

3. Environmental Factors

Evidence show that poverty, malnutrition, and inadequate learning experiences that include poor teaching and lack of instructional materials are causes of environmental disadvantage that make children prone to learning problems. Three types of environmental influences believed to be related to children's learning problems are: (Lovitt, 1978)

- a. *Emotional disturbance*
- b. *Lack of motivation*
- c. *Poor instruction*

Assessment of Learning Disabilities

1) *Norm-referenced Tests*. Students with learning disabilities manifest deficits in the expected number of skills for mastery in a particular grade level. The examples of this test are the National Achievement Test, the Regional Assessment Test and the School-based Achievement Test.

2) *Criterion-referenced test* are used to determine the mastery level of a predetermined criterion that the student should be capable of achieving test in specific subjects such as Reading, Language and Mathematics.

3) *Informal Reading Inventory* consists of a series of progressively more difficult sentences and paragraphs for oral reading. The student's mistakes in reading skills are recorded particularly in pronunciation of vowels and consonants, omissions, reversals, substitutions, and comprehension.

4) *Direct Daily Measurement* is a useful method of determining student's performance in learning a particular skill. Direct daily measurement of the mastery of the skill being taught informs the teacher about the deficit in learning and allows him or her to make the necessary adjustments in the lesson.

Attention Deficit Disorder (ADD) and Attention Deficit/Hyperactivity Disorder (ADHD)

Attention Deficit Disorder (ADD) and *Attention Deficit/Hyperactivity Disorder (ADHD)* are conditions in which children exhibit significant differences in the ability to pay attention and to engage in active work compared to their normal peers. These children show lack of control in saying things, cannot wait for their turn, and often engage in dangerous activities.

The *Diagnostic and Statistical Manual on Mental Disorders IV* of the American Psychiatric Association DSM IV, APA gives the symptoms and states that either of 1 or 2 are present.

1. Six or more of the following eight symptoms of inattention have persisted for at least six months:
 - a. Often fails to give close attention to details or makes careless mistakes in schoolwork, work, or other activities.
 - b. Often has difficulty sustaining attention in tasks or play activities.
 - c. Often does not follow through on instructions and fails to finish schoolwork, chores or duties in the workplace.
 - d. Often has difficulty organizing tasks and activities.
 - e. Often avoids dislikes or is reluctant to engage in tasks that require sustained mental effort such as schoolwork or homework.
 - f. Often losses things necessary for tasks or activities such as toys, school assignments, pencils, books, or tools.
 - g. Is often easily distracted by extraneous stimuli.
 - h. Is often forgetful in daily activities.
2. Six or more of the eight symptoms of hyperactivity-impulsivity hyperactivity:
 - a. Often fidgets with hands or feet or squirms in seat.
 - b. Often leaves seat in classroom or in other situations in which remaining seated is expected.
 - c. Often runs about or climbs excessively in situations in which it is inappropriate. In adolescents and adults this is an expression of subjective feelings of restlessness.
 - d. Often has difficulty playing or engaging in leisure activities quietly.
 - e. Is often on the go of often acts as if driven by a motor.

Application:

Answer the following questions on how you have learned about mental retardation.

1. In what way or ways is learning disabilities different from mental retardation?

2. Enumerate the learning and behavior characteristics of students with learning disabilities?

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output.

Criteria	5	3	2	1
Focus and Details	There is one clear, focused topic. Main idea is clear and well supported by detailed and accurate information.	There is one clear, well focused topic. Main idea is clear but not well supported by detailed information.	Main idea is somewhat clear.	The topic and main idea are not clear.
Organization	The introduction states the main topic, and provides an overview of the essay. Information is relevant and presented in a logical order. The conclusion is good.	The introduction states the main topic and provides an overview of the essay. A conclusion is included.	The introduction states the main topic. A conclusion is included.	There is no clear introduction, structure, or conclusion.
Word Choice	It uses vivid words and phrases. The choice and placement of words seems accurate, natural, and not forced.	It uses vivid words and phrases. The choice and placement of words is inaccurate at times and/or seems overdone.	It uses words that communicate clearly, but the writing lacks variety.	It uses a limited vocabulary. Jargon or clichés may be present and detract from the meaning.
Sentence structure, grammar and mechanics	All sentences are well constructed and have varied structure and length. There are no errors in grammar, mechanics, and/or spelling.	Most sentences are well constructed and have varied structure and length. There are few errors in grammar, mechanics, and/or spelling, but they do not interfere with understanding.	Most sentences are well constructed, but they have a similar structure and/or length. There are several errors in grammar, mechanics, and/or spelling that interfere with understanding.	Sentences sound awkward, are distractingly repetitive, or are difficult to understand. There are numerous errors in grammar, mechanics, and/or spelling that interfere with understanding.

Adapted from: <http://www.readwritethink.org/files/resources/printouts/Essay%20Rubric.pdf>

Summary of the Lesson:

Learning Disabilities refers to a heterogeneous group of disorders manifested by significant difficulties in the acquisition and use of listening, speaking, reading, writing, reasoning, or mathematical abilities. These disorders are intrinsic to the individual and presumed to be due to central nervous system dysfunction. Learning disabilities may appear across the life span. Problems in self-regulatory behavior, social perception and social interaction may exist in learning disabilities but do not themselves constitute a learning disability.

The *group of disorders is heterogeneous* indicates that there is not only one but several disorders that occur at the same time. No two learning disabled persons are alike in mental and behavioral characteristics.

The phenomenon is explained by the factor called *IQ achievement discrepancy*. The child does not learn knowledge and skills in accordance with his or her potential to learn as measured by a standardized mental ability test. While his or her mental ability is appropriate to his or her chronological

age, the child's performance in school shows that achievement is two or more years below the grade level.

Learning disabilities is *intrinsic* to the individual. This means that the causes of learning disabilities are organic, biological, genetic, or environmental. The organic and biological factors are traced to the central nervous system, particularly the brain. In most cases, the cause of learning disabilities remains a mystery.

The causes of learning disabilities are attributed to genetic and environmental factors. The genetic factors refer to the characteristics that are inherited through the genes, chromosomes, and deoxyribonucleic acid or DNA.

Learning disabilities is a complex condition. Therefore, a battery of three to five tests are used to identify students who may have learning disabilities. These are norm-referenced tests, process tests, informal reading inventories, criterion-referenced tests, and direct daily measurement of learning.

Assessment:

Multiple Choice: Choose the letter of the correct answer. Write your answers on the space provided.

- ____ 1) It is a generic term that refers to a heterogeneous group of disorders manifested by significant difficulties in the acquisition and use of listening, speaking, reading, writing, reasoning, or mathematical abilities.

A. Mental Retardation	C. Learning Disabilities
B. Learning Abilities	D. ADHD
- ____ 2) There are pieces of evidence of a discrepancy score of two or greater than two in intellectual ability and achievement in one or more of the following areas EXCEPT:

A. Oral expression	C. reasoning
B. Written expression	D. scientific calculation
- ____ 3) This refers to a disturbance in the ability to learn in general and the ability to learn to read in particular.

A. Dyslexia	C. ADHD
B. Mental Retardation	D. Learning Disability
- ____ 4) If the student poses severe problems in handwriting, spelling, composition, and writing, the students have a problem in

A. Reading	C. Spoken Language
B. Written Language	D. Behavior
- ____ 5) The following are the criteria that must be present in assessing children with learning disabilities EXCEPT:

A. Exclusion or absence of mental retardation, sensory impairment, and other disabilities
B. IQ score seemed to be distributed throughout the population according to the normal curve
C. Need for special education services
D. Severe discrepancy between the child's potential and actual achievement
- ____ 6) This is the characteristics of children with learning disabilities who cannot pay attention and

cannot focus on the teaching episode for a particular subject resulting to inefficient learning or no learning at all.

- A. Poor Memory
B. Attention Deficits
C. Social Interaction
D. Dyslexia

____ 7) This is a graphic measure and recording of the brain's electrical impulses. A digitally computerized recording and analysis of the brain waves.

- A. EEG
B. CT scan
C. MRI
D. X-ray

____ 8) Which of the following is NOT an environmental influence believed to be related to children's learning problem according to Lovitt (1978)?

- A. poor memory
B. Emotional disturbance
C. lack of motivation
D. poor instruction

____ 9) These are examples of norm-referenced test EXCEPT

- A. National achievement Test
B. Regional Assessment Test
C. Informal Reading Inventory
D. School-based Achievement Test

____ 10) This is a condition in which children exhibit significant differences in the ability to pay attention and to engage in active work compared to their normal peers.

- A. Dyslexia
B. ADHD
C. Mental Retardation
D. Learning Disability

Enrichment Activity:

After gaining knowledge about learning disabilities, what do you think should have been done by the school and the teachers to help students with learning disabilities?

Directions: Your essay will be graded based on this rubric. Consequently, use this rubric as guide in writing your essay and check it again before submitting your final output.

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Suggested Links (Optional):

- https://www.naset.org/fileadmin/user_upload/LD_Report/Issue__3_LD_Report_Characteristic_of_LD.pdf

References/Attributions:

- Inciong, Teresita G., Quijano, Yolanda S. and Capulong, Yolanda T. Introduction to Special Education. Manila, Philippines: Rex Bookstore, 2007.